

Serotonin syndrome induced by Bromfed DM in a patient on sertraline

Serotonin syndrome induced by Bromfed DM in a patient on sertraline

Published case report: PMC12638710 / PMID 41282384

Source: <https://pmc.ncbi.nlm.nih.gov/articles/PMC12638710/>

License: CC BY

License evidence: <https://www.ncbi.nlm.nih.gov/pmc/utils/oa/oa.fcgi?id=PMC12638710>

Dataset note: This case section was extracted mechanically from the cited PMC article. No clinical facts were added.

Case presentation

A 52-year-old male with a medical history of hyperthyroidism, hypertension, and generalized anxiety disorder had been on sertraline 50 mg daily since September 2023. Other medications included propranolol 10 mg daily, methimazole 10 mg daily, and olmesartan-hydrochlorothiazide (HCTZ) 40–25 mg daily.

Four days prior to the office visit, the patient presented to urgent care for a persistent cough. He tested negative for COVID-19, influenza, and streptococcus and was prescribed Bromfed DM syrup (brompheniramine 2 mg, pseudoephedrine 30 mg, dextromethorphan 10 mg per 5 mL) to be taken 5 mL every 6 hours, along with a methylprednisolone dose pack.

Three days after initiating Bromfed DM, the patient developed dizziness, confusion, photophobia, irritability, difficulty with speech articulation, and noticeably dilated pupils. He also reported a sensation of increased eye pressure, which he described as “eye bulging,” that impaired his ability to drive safely. His wife observed spontaneous foot movements during sleep, which were consistent with myoclonic jerks.

The patient and his wife independently researched his symptoms, suspected SS, and discontinued both sertraline and Bromfed DM a day before his primary care appointment.

On examination at that appointment, he reported mild improvement. Vitals were normal (BP: 106/57, HR: 70 bpm, Temp: 36.7°C). He had intact neurological function, normal gait, no hyperreflexia, and no clonus. No laboratory or imaging studies were deemed necessary at this time, as the clinical presentation was consistent with resolving SS and the patient’s condition was improving. However, he continued to experience disorientation and cognitive impairment in the absence of any alcohol use.